Cash Transfers, Health, and the Transition to Adulthood

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University of Helsinki & Helsinki GSE

Health problems among adolescents are one of the big issues for high-income countries



The New York Times

Surgeon General Warns of Youth Mental Health Crisis

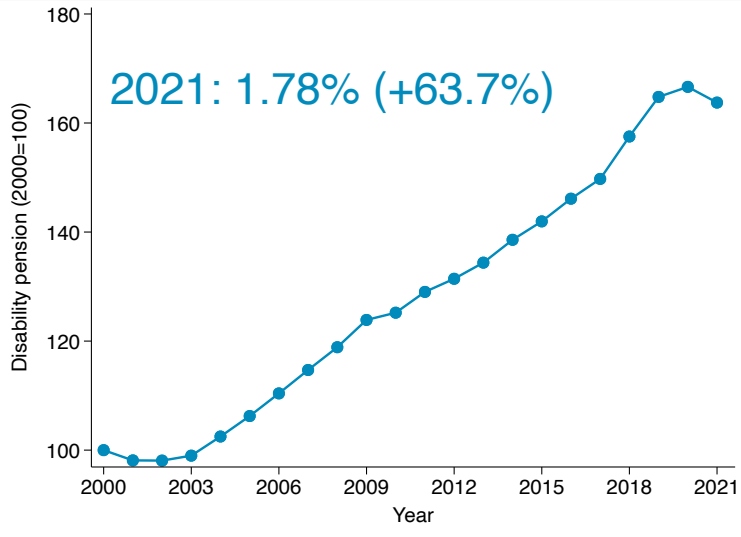
The coronavirus pandemic intensified a rise in adolescent depression, anxiety and mental health distress that was underway before the spring of 2020.

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Health problems among adolescents are one of the big issues for high-income countries



Disability on the rise: Finland



Share of under 30-year-olds, Year 2000=100

What can policy do for adolescents with health problems?

- Traditional social policies: cash transfers to families, health insurance, tuition-free education
- Despite the low cost of education, many drop out and turn to disability benefits

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- Traditional social policies: cash transfers to families, health insurance, tuition-free education
- Despite the low cost of education, many drop out and turn to disability benefits
- **This paper: Paying adolescents to attend education (or other rehabilitation)**
  - A temporary transfer, ages 16-20
  - Requires a health problem that threatens their education or employment (ranging from intellectual disability to depression, ADHD, and arthritis)
- Rationale: instead of passive disability benefits, acquire human capital, which will help in the labor market

In this paper

What are the long-run effects of paying adolescents with health problems to attend education?

- Data on applications, labor market outcomes until age 30

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Marginal applicants Examiner IV	**Employment** -11 **pp**	No increase in education Less work

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Population / design	After the benefit	Potential mechanism
Marginal applicants Examiner IV	**Employment** -11 **pp**	No increase in education Less work
Lowest labor-market prospects Selection on observables	**Employment** $+4.7$ **pp**	Passive welfare $\rightarrow$ education

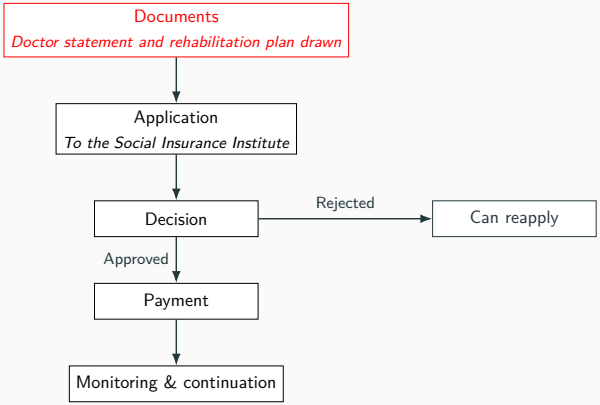
Contribution

- First evidence on effects of a health-based schooling transfer for adolescents
- Previous work on
  - On disability benefits with no education incentives (Deshpande, 2016; Levere, 2021) $\Rightarrow$ A benefit with human capital condition can also have negative effects
  - Education/activation benefits for inactive youth (Dearden et al., 2009; Britton et al., 2025; Filippucci, 2025) $\Rightarrow$ A health based program
  - ALMPs and rehabilitation policies (Markussen, 2012; Fogelgren et al., 2023; Reiterä, 2021; Humlum et al., 2025; Borgan Reiling et al., 2024) $\Rightarrow$ This policy changed income and incentives, not type of rehabilitation

Youth Rehabilitation Allowance (YRA)

- **Benefit:** Around €700/month, conditional on attending education or another rehabilitative activity, for ages 16–20
- **Eligibility and conditions:** Health screening; not means-tested; no need to document expenses; can live with parents
- **Context:** Finland has no tuition, universal health insurance, and smaller means-tested benefits. Disability Insurance is a permanent program of similar size, available from age 16

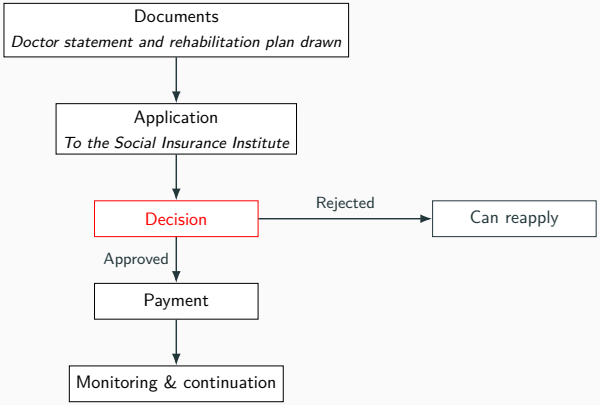
Application process



Required for an application

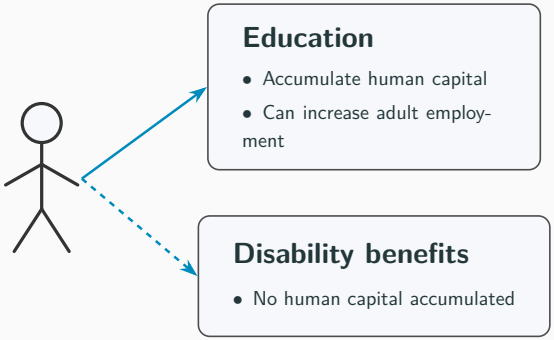
- Doctor statement and a plan of rehabilitation with the school and/or social services

Application process

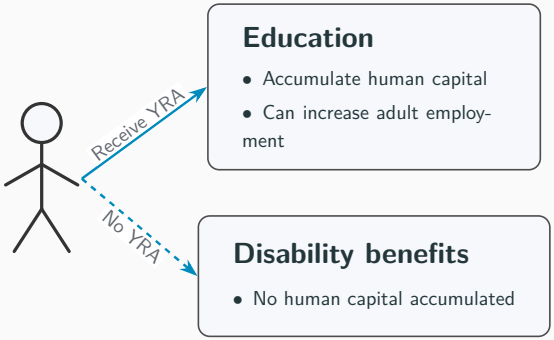


- Applications allocated to examiners from a running queue => essentially random
- Examiners assess the severity of the disability, whether the plan is adequate, etc.

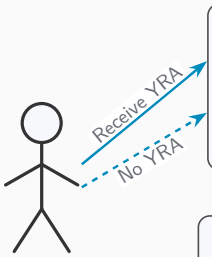
Simple conceptual framework: decisions at age 16



Simple conceptual framework: decisions at age 16



Decisions at age 16 if the schooling condition is not binding



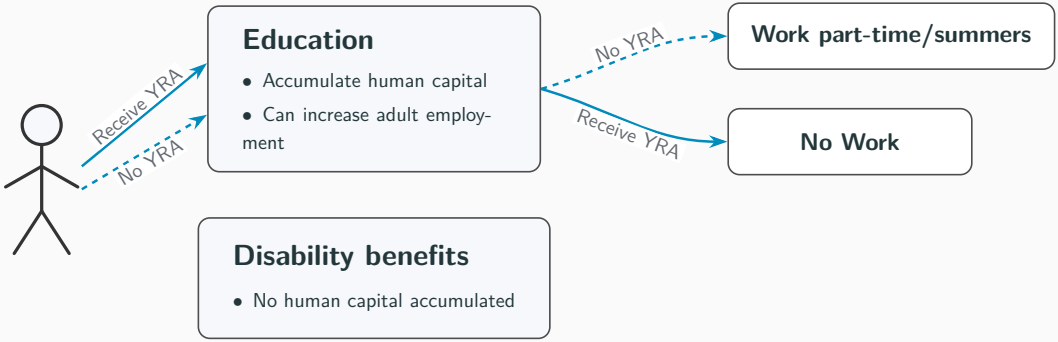
Education

- Accumulate human capital
- Can increase adult employment

Disability benefits

- No human capital accumulated

Decisions at age 16 if the schooling condition is not binding

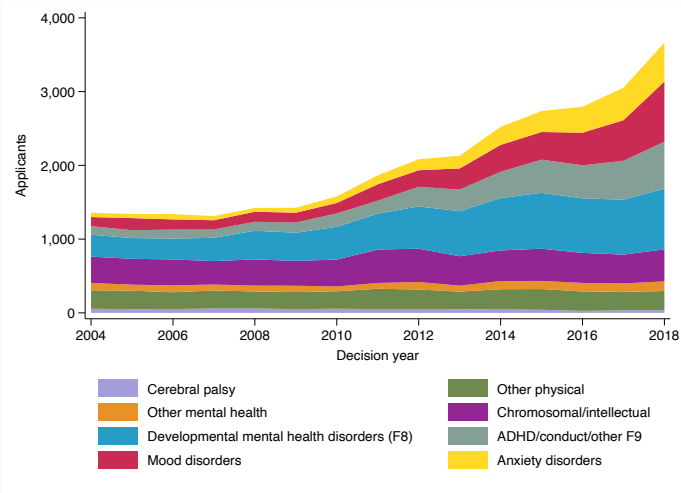


Data

Individual data on the entire population of Finland

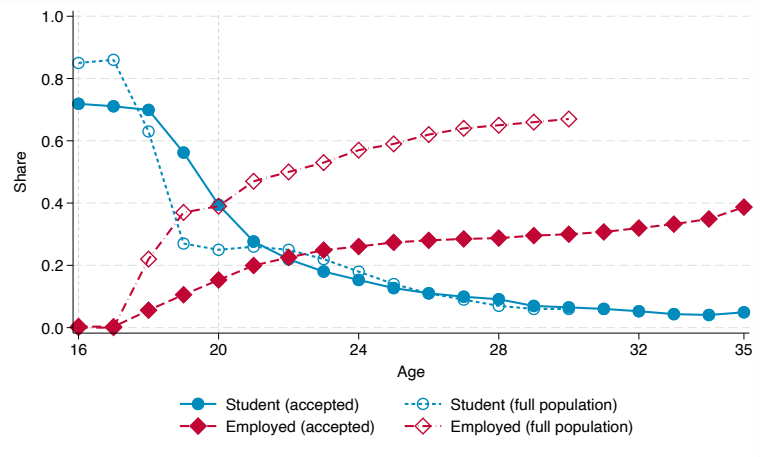
- Applications (N=30,623) from 2004-2018, including examiner IDs, diagnoses, and dates
- Labor market, demographics, education, other benefits
- Healthcare use from the public health system (most healthcare use for this population) and medical prescriptions
- Criminal convictions, police reports, enforced debts

Large increase in applications driven by mental health conditions



Summary statistics

Applicants have poor labor market outcomes



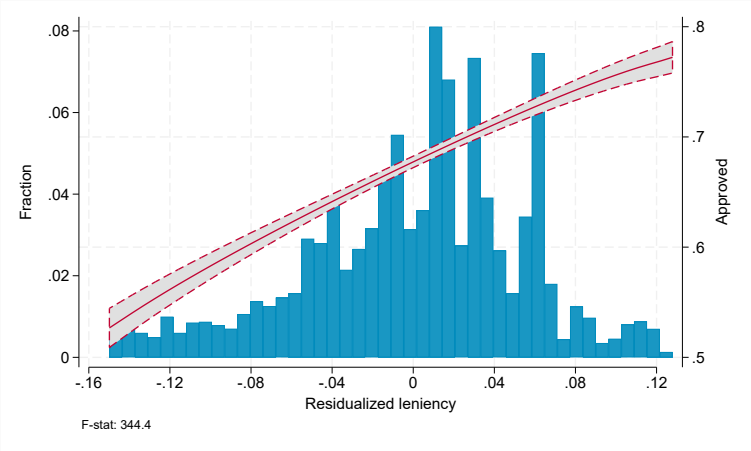
End-of-year activity. Full population reweighted to the same cohorts as the applicants.

Methods: Examiner IV

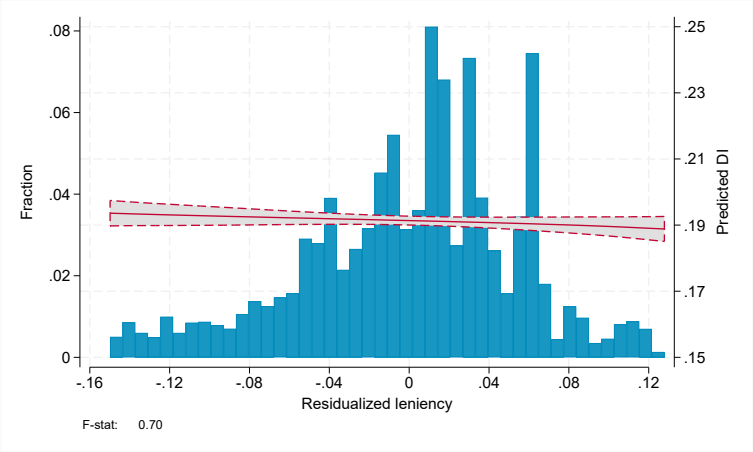
- Applications allocated to examiners from a running queue => essentially random within time $\times$ office $\times$ language
- I use the (leave-out) approval tendency of the examiner as an instrument (Examiner/Judge-IV), shrink using EB
- First stage outcome/treatment: first application approved Ever accepted
- Assumptions needed for LATE: relevance, exogeneity, exclusion, monotonicity

EB details Illustration

The instrument strongly predicts approval

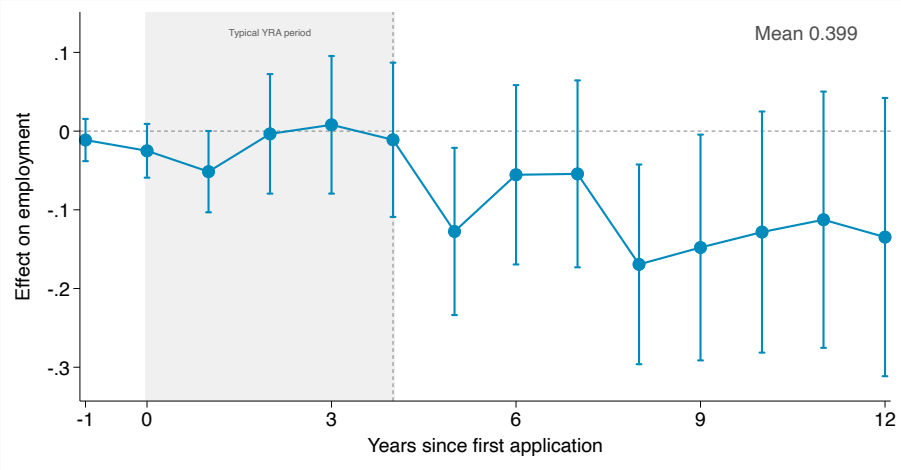


The instrument is uncorrelated with background characteristics



Balance coefficients (p=0.62) | Weak IV | Monotonicity | Split-sample monotonicity

IV: After the benefit ends, employment falls



After the benefit: Reduced labor market participation

Years 6–11 after application	IV	Mean
Employed	-0.112**	0.399
	(0.050)	
Student	-0.006	0.147
	(0.032)	
NEET	0.117**	0.454
	(0.054)	
Empl. income	-1,756	9,177
	(1,331)	
Individuals	23,596	
First-stage F	315	

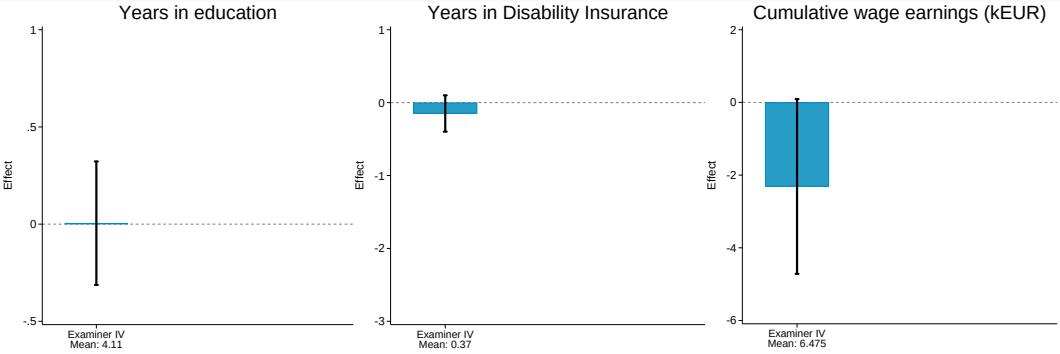
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- Negative effects throughout
- Robust to different specifications and choices

With additional controls | Different specifications | Different case limits | Area leave out

During the benefit: Little displacement from disability benefits to education



Cumulative effects until age 21.

Beyond IV?

- The youth rehabilitation allowance did not move the marginal applicants from disability benefits to education
- Could it help individuals with the lowest labor market prospects?
  - Rest of the talk, 3 groups: marginal applicants (IV compliers) + low labor market prospects and moderate labor market prospects
  - In practice: predict DI recipiency using background characteristics, classify the top 20% with the highest likelihood as low labor market prospect sample
- Most of the low labor market prospect sample are not marginal $\rightarrow$ the IV has poor power

Methods: Selection-on-observables

- I complement the IV with selection-on-observables: more precise estimates for subgroups underrepresented among compliers

Assumptions

- Conditional on observables, treatment assignment is unrelated to potential outcomes
- No observables perfectly predict treatment

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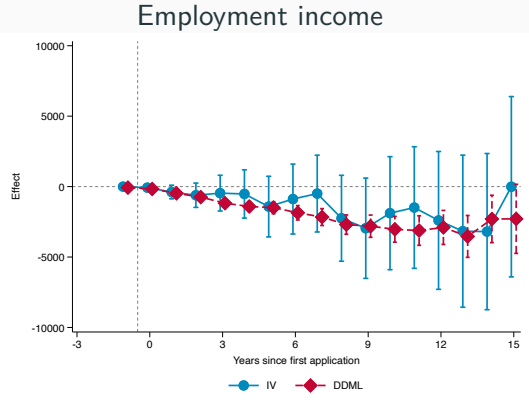
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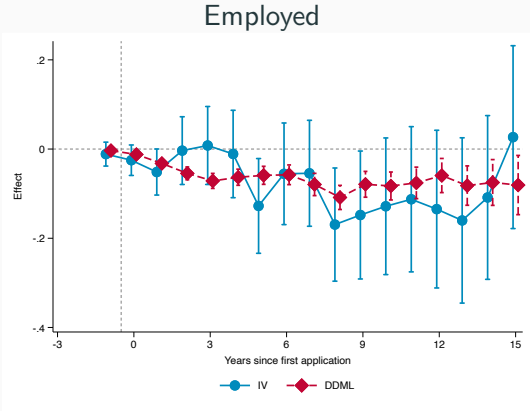
- Conditional on observables, treatment assignment is unrelated to potential outcomes
- No observables perfectly predict treatment

Why plausible here?

- Approved and rejected applicants are relatively similar (Bound, 1991)
- Rich set of covariates; use Double/Debiased Machine Learning (DDML) (Chernozhukov et al., 2018; Chetty et al., 2026) DDML estimator Covariates
- Assess the identifying assumption in multiple ways

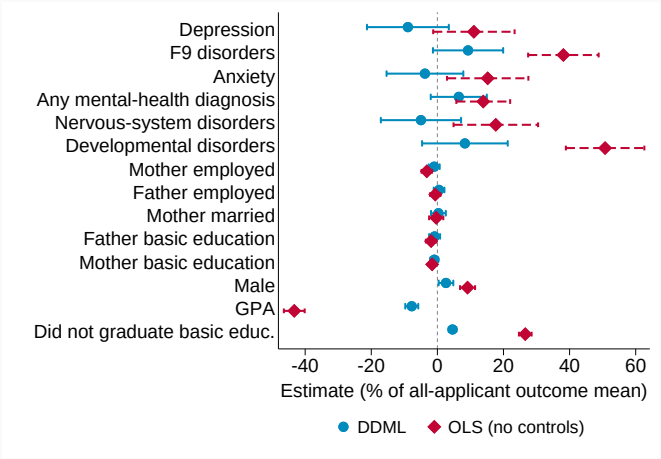
Validating the comparison using the IV





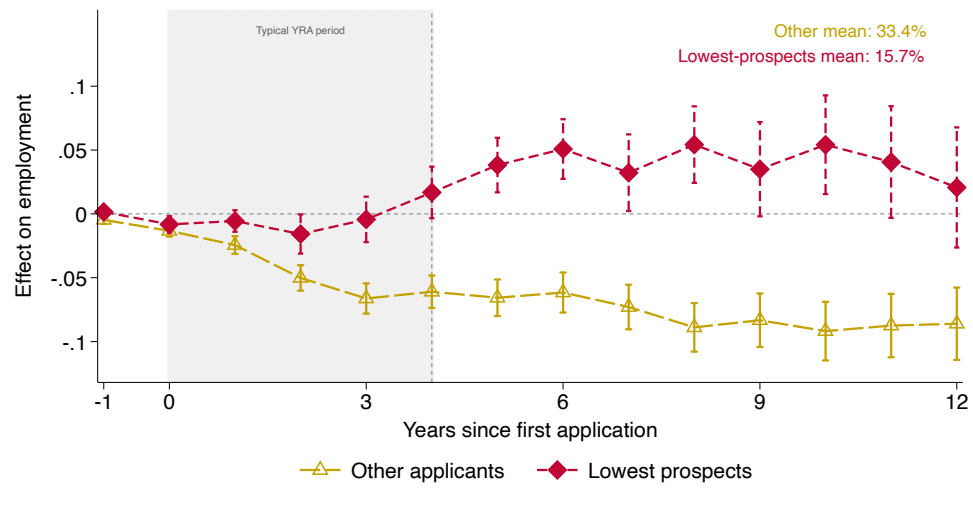
Low-prospect placebos | Overlap | Low-prospect overlap

Selection-on-observables: Placebo outcomes



Notes: Entire domain of covariates left out.

Selection-on-observables: positive effects for low labor market prospect sample



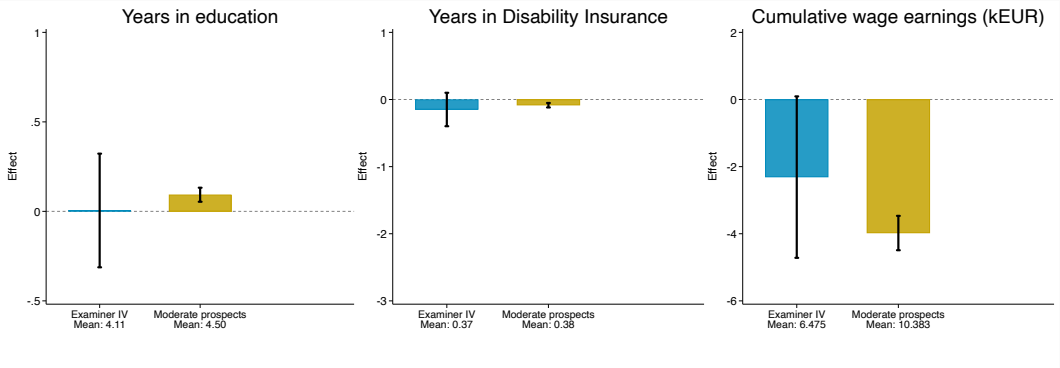
Overall negative effects, but positive for the lowest prospect sample

	Employed	NEET	Employment income (euros)
Moderate prospects			
N = 79,592	-0.074***	0.073***	-2,306***
	(0.004)	(0.004)	(95)
Oster delta(0)	5.2	5.3	6
Outcome mean	.33	.51	7,951
Lowest prospects			
N = 22,997	0.047***	-0.049***	679***
	(0.005)	(0.007)	(114)
Oster delta(0)	-41	-28	-12
Outcome mean	.16	.77	2,826
Overall			
N = 102,589	-0.047***	0.046***	-1,632***
	(0.003)	(0.003)	(77)
Oster delta(0)	3.1	2.6	3.7
Outcome mean	.29	.57	6,802
Controls	All	All	All

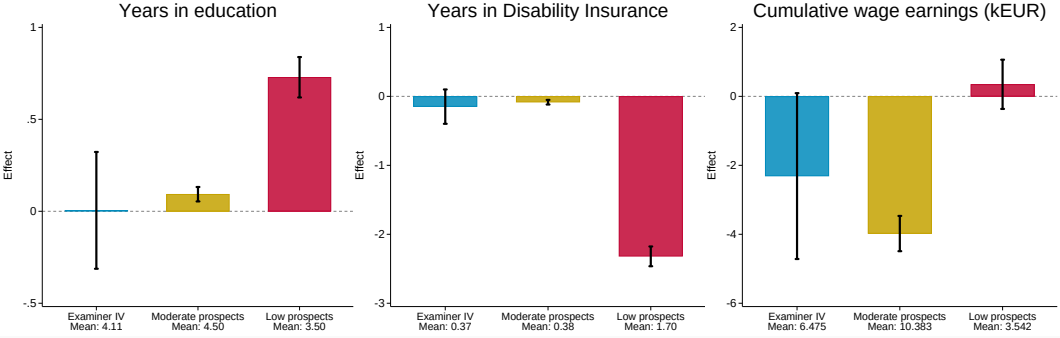
Marginal treatment effects

Notes: Oster delta from linear regression.

Moderate prospect sample: little gains in education, working falls



Low prospect sample: moved from welfare to education



Conceptual framework: Human capital and adult employment

YRA approval (d) can change schooling and early work between ages 16 and 20.

$$H_i^A(d) = h(G_i) + \underbrace{\theta_S S_i(d)}_{\text{education}} + \underbrace{g(X_i(d))}_{\text{early work}}, \qquad \theta_S \geq 0, \quad g' > 0, \quad g'' < 0.$$

G_i: ninth-grade GPA S_i: later education X_i: early work

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Adult employment E_i^A requires the offered wage to exceed the participation threshold:

$$\log w_i^*(d) = \alpha + \lambda H_i^A(d) + u_i, \qquad \lambda > 0$$
$$E_i^A(d) = \mathbf{1}\{\log w_i^*(d) \geq R_i\}.$$

Higher human capital raises the offered wage and can move an applicant into employment.

Conceptual framework: Human capital and adult employment

- With linear approximation, the effect of the Youth Rehabilitation Allowance on employment probability is

$$\underbrace{\Delta p}_{\text{change in employment}} \approx \underbrace{a\theta_S \Delta \bar{S}}_{\text{education contribution}} + \underbrace{a\Delta \bar{g}}_{\text{early-work contribution}}, \qquad a \geq 0.$$

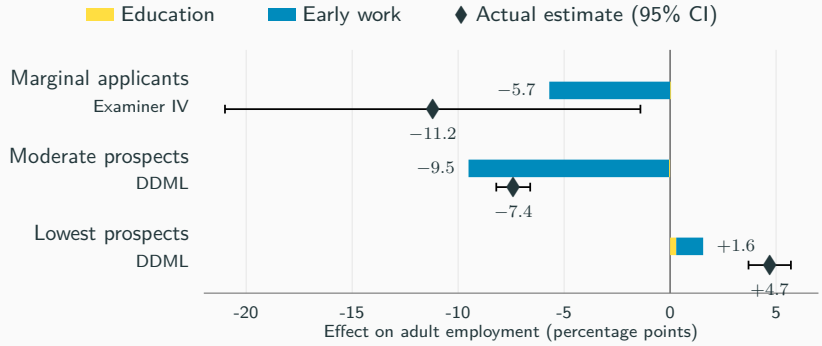
- Gives clear predictions
- A simple model of how human-capital accumulation in adolescence affects later employment
- Does not rule out other mechanisms

Derivations

Education and early-work changes reproduce the sign pattern

$$\widehat{\Delta p_k}^{\,\text{mean}} = \widehat{\beta}_{S,k}\widehat{\Delta \bar{S}}_k + \widehat{\beta}_{X,k}\widehat{\Delta \bar{X}}_k \quad + \widehat{\beta}_{X^2,k}\left[(\bar{X}_{0k} + \widehat{\Delta \bar{X}}_k)^2 - \bar{X}_{0k}^2\right].$$

Descriptive projection evaluated at each group's mean early-work exposure.



Regression Work attachment and adult employment

Other mechanisms tested

- 'Welfare cultures' (Dahl et al., 2014): information or beliefs changed, stigma etc
  - Hard to measure in administrative data
  - Indirect test: Does approval increase future recipiency of other health-based benefits
  - Answer: No for disability insurance and sick pay Other welfare benefits
- Adverse health effects due to e.g. consumption of harmful goods (Evans and Moore, 2011; Black et al., 2024; Chorniy et al., 2025)
  - Test using healthcare and crime data
  - No evidence of this Crime and health

Welfare effects

- No offsetting improvements on crime or health for the moderate prospect/marginal applicants
- The MVPF for marginal applicants is 0.83 MVPF
- For the lowest prospect group, the policy pays for itself
- $\Rightarrow$ Better screening could preserve the positive outcomes while getting rid of the negative

Conclusions

- Mainly negative labor-market effects after the benefit ends
- Positive effects among individuals who move from welfare to education

$\Rightarrow$ Programs can have unexpected effects

$\Rightarrow$ Design calls for careful consideration of the target population and what they would do without the program

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Links

- More outcomes Broader outcomes | Different welfare benefits | DDML More outcomes I | DDML More outcomes II
- Heterogeneity etc. More heterogeneity
- Robustness IV robustness | Ever accepted | IV/DDML comparison | Caseload limits | Leave-out area

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The employment effect through human capital

The difference between an individual with and without the Youth Rehabilitation Allowance is

$$\Delta H_i = \underbrace{\theta_S \Delta S_i}_{\text{education}} + \underbrace{\Delta g_i}_{\text{early work}}, \qquad \Delta g_i \equiv g(X_i(1)) - g(X_i(0)).$$

Δ denotes the treated-minus-untreated value of each component.

A linear approximation to the employment probability gives

$$\Delta p \approx \underbrace{a\theta_S \Delta \bar{S}}_{\text{education contribution}} + \underbrace{a\Delta \bar{g}}_{\text{early-work contribution}}, \qquad a \geq 0.$$

$\Delta p = \mathbb{E}[p_i(1) - p_i(0)]$; bars denote average treatment-induced changes.

Back

From human capital to employment

Let m_i denote the slope of applicant i's employment probability at the baseline participation threshold. A first-order approximation gives

$$\Delta p \approx \mathbb{E}[m_i \lambda \Delta H_i] \,.$$

If $\mathrm{Cov}(m_i, \Delta H_i) = 0$, then

$$\Delta p \approx \underbrace{a\theta_S \Delta \bar{S}}_{\text{education channel}} + \underbrace{a\Delta \bar{g}}_{\text{early-work channel}} , \qquad a = \lambda \mathbb{E}[m_i] \geq 0.$$

- Education raises human capital through $\theta_S \Delta \bar{S}$.
- Early work raises human capital through $\Delta \bar{g}$.
- Health, work costs, and adult benefits instead shift the threshold R_i.

Back

Empirical implementation of the channels

Among initially rejected applicants, estimate the predictive employment gradient

$$E_i^A = \beta_0 + f(G_i) + \beta_S S_i + \beta_X X_i + \beta_{X^2} X_i^2 + C_i'\gamma + \varepsilon_i.$$

For each group k, combine these gradients with the treatment-induced changes:

$$\widehat{\Delta p_k}^{\text{proj}} = \widehat{\beta}_S \widehat{\Delta \bar{S}}_k + \widehat{\beta}_X \widehat{\Delta \bar{X}}_k + \widehat{\beta}_{X^2} \widehat{\Delta \overline{X^2}}_k.$$

- S_i and X_i are cumulative schooling and earnings at ages 16–20.
- The β coefficients are predictive, not causal returns.
- Compare each projection with the group's estimated adult-employment effect.

Back

Regression behind the accounting exercise

$$E_i^A = \beta_{0,k} + f_k(G_i) + \beta_{S,k} S_i + \beta_{X,k} X_i + \beta_{X^2,k} X_i^2 + C_i' \gamma_k + \varepsilon_i.$$

	Marginal (compliers)	Moderate (applicant gradient)	Lowest (low-prospect sample)
$\widehat{\beta}_{S,k}$: years in education	-0.90^{***}	-0.50^{***}	0.40
	(0.30)	(0.20)	(0.30)
$\widehat{\beta}_{X,k}$: early earnings	2.90^{***}	3.15^{***}	4.15^{***}
	(0.15)	(0.11)	(0.27)
$\widehat{\beta}_{X^2,k}$: early earnings squared	-0.041^{***}	-0.046^{***}	-0.069^{***}
	(0.003)	(0.003)	(0.006)
Observations	4,670	9,506	2,224

Outcome: adult employment (pp). S_i: years in education; X_i: cumulative earnings through age 20 (EUR 1,000). Standard errors in parentheses. Predictive, not causal. *** $p < 0.01$.

Back

Summary statistics

Back

	(1) Gen. pop.	(2) Approved	(3) Rejected
Male	0.52	0.55	0.50
Student	0.98	1.00	1.00
Employed	0.02	0.00	0.00
Healthcare visits	0.95	8.34	6.51
Anxiety Diagnosis	0.01	0.09	0.08
Depression diagnosis	0.01	0.10	0.08
Other MH diag.	0.03	0.37	0.29
Single parent HH	0.21	0.29	0.30
Urban	0.81	0.78	0.80
Mother age	44	44	44
Father age	46	47	47
Mother DI	0.03	0.08	0.07
Father DI	0.04	0.08	0.08
Mother married	0.70	0.56	0.56
Mother employed	0.80	0.68	0.70
Father employed	0.78	0.67	0.68
Mother taxable income	24912	23961	24470
Father taxable income	34873	32942	33267
Observations	1421282	20830	9793

Empirical Bayes shrinkage Back

$$\hat{\theta}_{ij} = \frac{1}{N_j - 1}\left(\sum_{k=1}^{N_j} Approved^*_{jk} - Approved^*_{it}\right),$$

$$\theta^*_{ij} = \left(\frac{\sigma^2_\theta}{\sigma^2_\theta + s^2_j}\right)\hat{\theta}_{ij} + \left(\frac{s^2_j}{\sigma^2_\theta + s^2_j}\right)\mu_\theta.$$

- New-ish innovation in Judge IV (Humphries et al., 2025; Frandsen et al., 2026)
- Idea: Leave-out mean a noisy measure of the true leniency. EB can improve the first stage
- Noisier examiner estimates receive more shrinkage toward the grand mean.
- The shrunk leave-out approval tendency is used as the instrument.

Balance coefficients: background

	Approved	EB IV
Parents		
Mother wage inc.	-0.000235	-0.0000463*
	(0.000203)	(0.0000274)
Father employed	-0.00189	-0.00154
	(0.00716)	(0.000968)
Applicant		
Month of dec.	-0.00433***	-0.0000106
	(0.000766)	(0.000104)
Male	0.0240***	0.000786
	(0.00551)	(0.000745)
GPA (9th grade)	-0.0219***	0.000104
	(0.00228)	(0.000308)
Age	-0.0470***	-0.000636
	(0.00337)	(0.000455)

* p<0.1, ** p<0.05, *** p<0.01.

Diagnoses and healthcare

Back

Balance coefficients: diagnoses and healthcare

	Approved	EB IV
Diagnosis in application		
Mood/Anx/Dev. dis.	-0.0841***	0.0000300
	(0.00774)	(0.00105)
ADHD/conduct	-0.117***	-0.00000438
	(0.0101)	(0.00137)
Other Physical	-0.146***	-0.0000604
	(0.00948)	(0.00128)
Healthcare use		
Hosp. visits	0.00129***	0.0000173
	(0.000199)	(0.0000268)
Any intell. dis. visit	0.0722***	0.000783
	(0.00777)	(0.00105)
N of diff. prescr.	-0.00114**	-0.00000734
	(0.000487)	(0.0000658)
Additional covariates included but not reported here		
Observations	30623	30623
F (all cov's)	52.56	0.899
p (all cov's)	3.24e-305	0.624

* p<0.1, ** p<0.05, *** p<0.01.

Background

Back

Weak-instruments I

- Main-specification first-stage F-statistic: 344.
- Examiner-design F-statistics are not directly comparable to the usual single-instrument rule of thumb.
- Anderson–Rubin confidence intervals remain valid under weak instruments and are close to the conventional intervals.

Back

Split-sample monotonicity check

Back

	First application approved	N	F
Female [Male]	0.412***	14301	83.05
	(0.0453)		
Male [Female]	0.365***	16080	88.72
	(0.0388)		
Int./chrom. dis. [Others]	0.117***	26549	48.93
	(0.0168)		
Mood/anx. [Others]	0.181***	22722	57.81
	(0.0238)		
Dev. dis. [Others]	0.296***	20562	71.52
	(0.0351)		
Physical [Others]	0.229***	18165	40.24
	(0.0361)		
High acc. rate [Low]	0.456***	9141	60.40
	(0.0587)		
Low acc. rate [High]	0.280***	21188	86.99
	(0.0300)		

Standard errors in parentheses

* $p < 0.1$, ** $p < 0.05$, *** $p < 0.01$

First stage outcome definition

	(1) First application approv.	(2) Ever in YRA	(3) Total YRA Benefits
EB IV	0.803***	0.547***	13792.6***
	(0.0433)	(0.0395)	(1330.2)
N	30623	30623	30623
F-Statistic	344.4	191.7	107.5

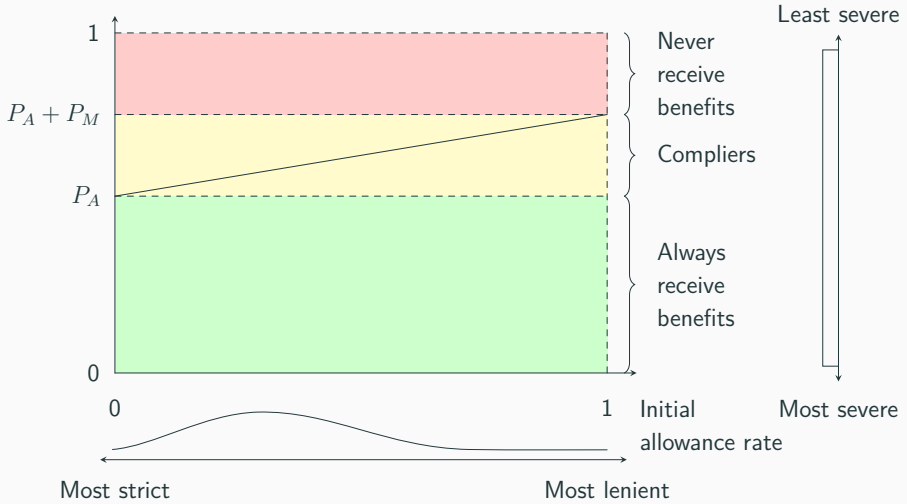
Standard errors in parentheses

* p<0.1, ** p<0.05, *** p<0.01

Back

Examiner IV illustrated

Back



(Adapted from Maestas et al. (2013))

Monotonicity across subgroups

Subsample	First-stage coefficient	Standard error	N
Male	0.785	0.058	16185
Female	0.819	0.065	14354
Urban	0.820	0.049	24353
Rural	0.689	0.094	6202
Mood disorder	0.887	0.121	4214
Anxiety disorder	0.964	0.153	2714
Physical condition	0.722	0.098	5964
Attention-deficit disorder	0.757	0.157	2375
Mother: higher education	0.758	0.080	9579
Mother: no higher education	0.826	0.052	20077
Decision year after 2007	0.791	0.053	23827
Finnish-speaking	0.820	0.044	29217

Back

Interactive DDML: implementation details

- **Target and assumptions.** ATE of initial approval D_i within each analysis sample; $(Y_i(1), Y_i(0)) \perp D_i \mid X_i$ and overlap.
- **Nuisance functions.** Estimate $m_d(X) = \mathbb{E}[Y \mid D = d, X]$ and $p(X) = \Pr(D = 1 \mid X)$. Short-stack OLS and gradient boosting for m_d; logit and gradient boosting for p.
- **Covariates and support.** Pre-application applicant, application, family, and healthcare data; diagnosis group $\times$ application year. Trim the top and bottom 1% of the estimated propensity score.
- **Cross-fitting.** Five applicant-clustered folds: train on four, predict the fifth, and keep all records for one applicant in one fold. Re-estimate nuisance functions for every outcome and analysis sample.

$$\widehat{\psi}_i = \widehat{m}_1(X_i) - \widehat{m}_0(X_i) + \frac{D_i\{Y_i - \widehat{m}_1(X_i)\}}{\widehat{p}(X_i)} - \frac{(1 - D_i)\{Y_i - \widehat{m}_0(X_i)\}}{1 - \widehat{p}(X_i)}, \qquad \widehat{\tau} = \frac{1}{N}\sum_i \widehat{\psi}_i.$$

- **Inference.** Influence-function standard errors clustered by applicant. Orthogonality and cross-fitting reduce nuisance-model bias; identification still requires conditional ignorability.

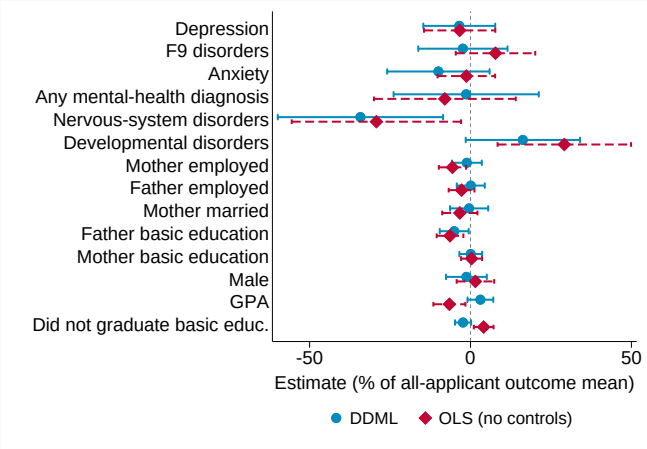
Back

DDML covariates

- **Applicant:** gender, age, ninth-grade GPA, region, and urban residence
- **Application:** application year and month, diagnosis group $\times$ application year
- **Family:** parent ages, education, employment, earnings, and Disability Insurance receipt
- **Healthcare:** visits by diagnosis group, prescriptions, drug expenses, and hospital days

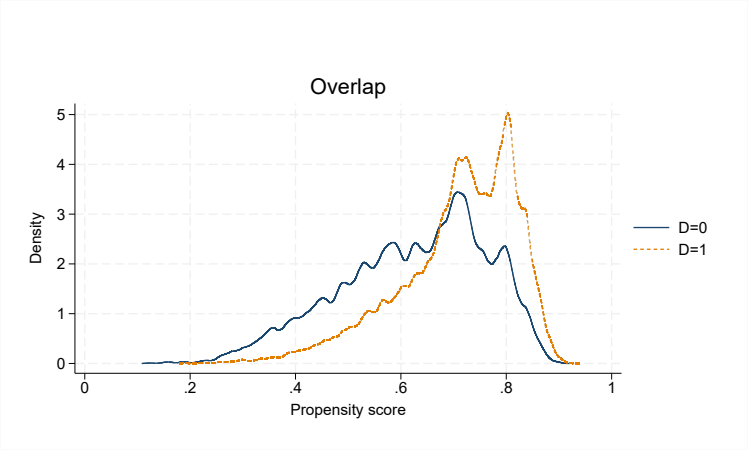
Back

DDML placebo outcomes: low labor market prospects



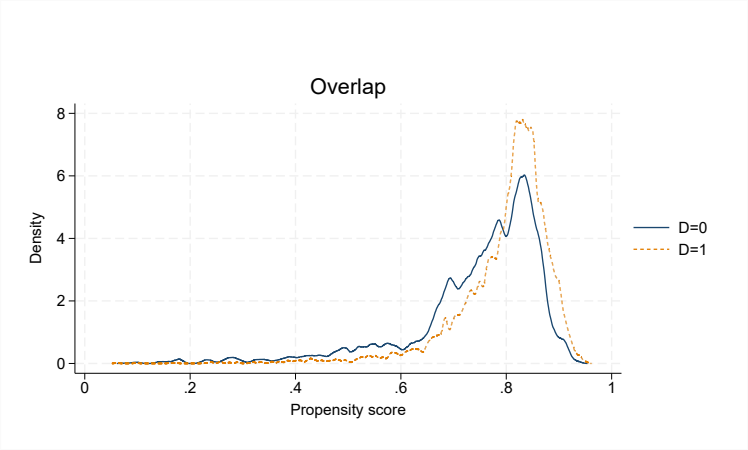
Back

DDML overlap



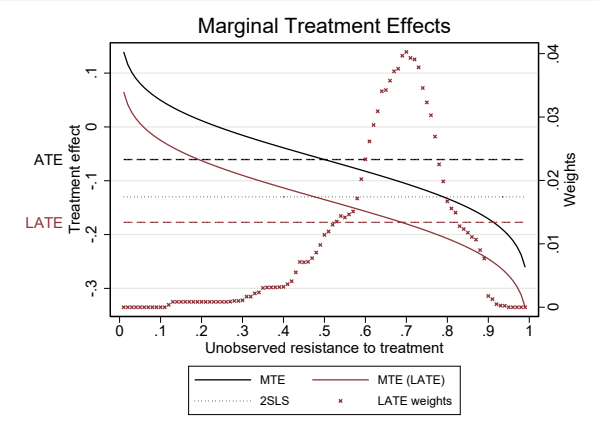
Back

DDML overlap: low labor market prospects



Back

Marginal treatment effects: normal specification



Notes: Outcome is employment in years 6–12 after application. LIV estimate using `mtefe`. Confidence intervals are omitted because they span most of the possible interval for the binary outcome in most specifications.

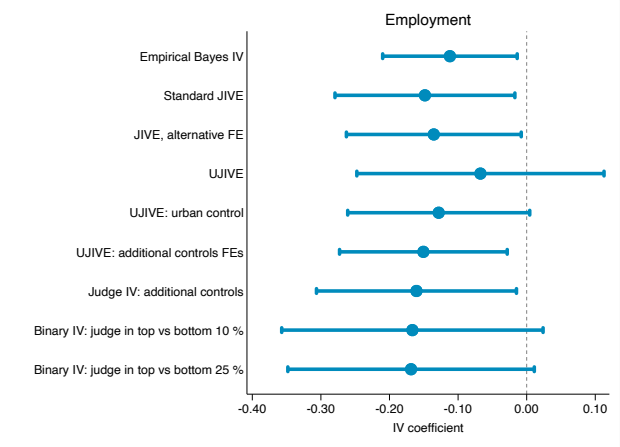
Back

Robustness I: Additional controls

	(1) NEET	(2) Employed	(3) Empl. income (euros)	(4) Student
Panel A. Main estimates				
OLS	0.115***	-0.100***	-3,083***	-0.015***
	(0.003)	(0.003)	(77)	(0.002)
IV	0.117**	-0.112**	-1,756	-0.006
	(0.054)	(0.050)	(1,331)	(0.032)
AR p-value	0.031	0.026	0.189	0.842
First-stage F	315	315	315	315
Panel B. IV with additional controls				
IV	0.100*	-0.098**	-1,455	-0.003
	(0.051)	(0.049)	(1,298)	(0.032)
AR p-value	0.052	0.043	0.263	0.927
First-stage F	319	319	319	319
Untreated complier mean	*0.454*	*0.399*	*9,177*	*0.147*
Sample mean	*0.567*	*0.295*	*6,802*	*0.138*
Observations	102,595	102,595	102,595	102,595
Individuals	23,596	23,596	23,596	23,596

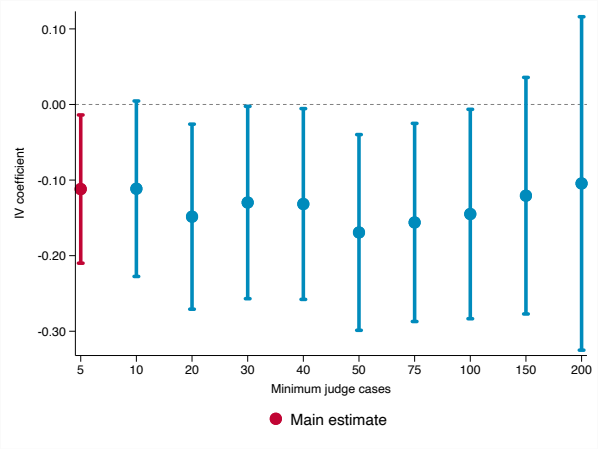
Back

Robustness II: Different specifications



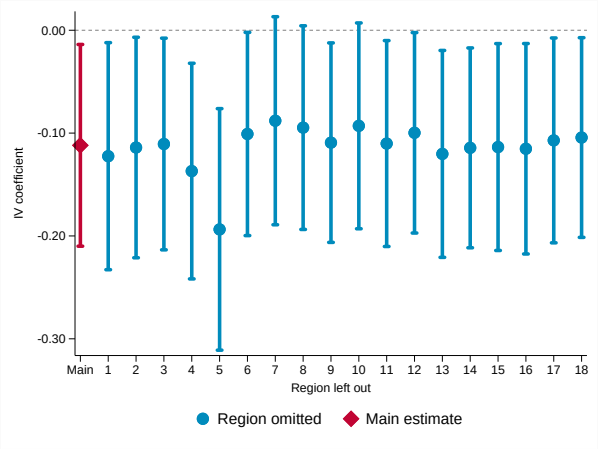
Back Conclusions

Robustness III: Different caseload limits



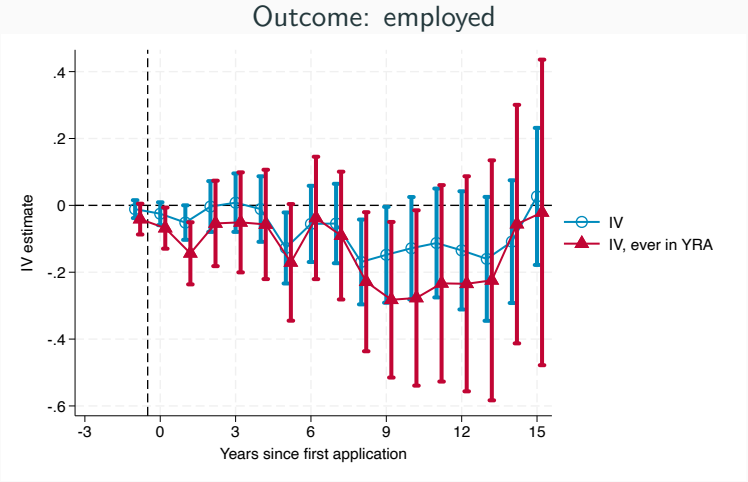
Back Conclusions

Robustness IV: Leave area out



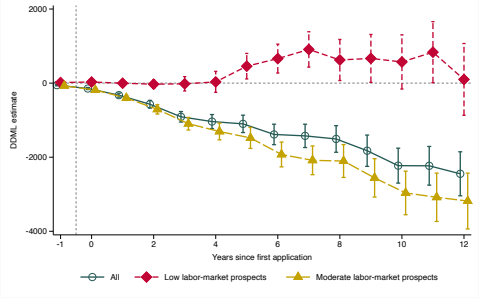
Back Conclusions

Using 'Ever approved to the benefit' as the endogeneous variable

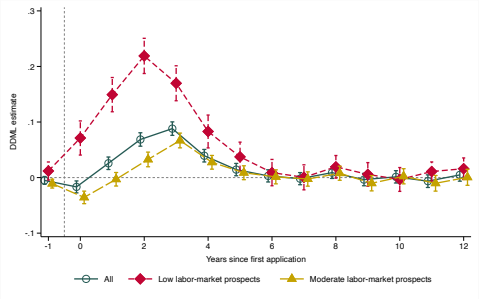


DDML: More outcomes I

Employment Income



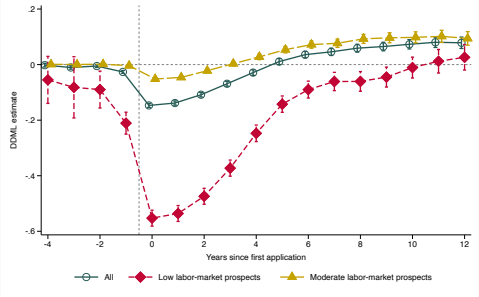
Student



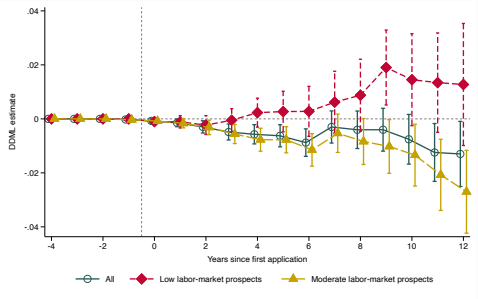
Conclusions

DDML: More outcomes II

Disability Insurance



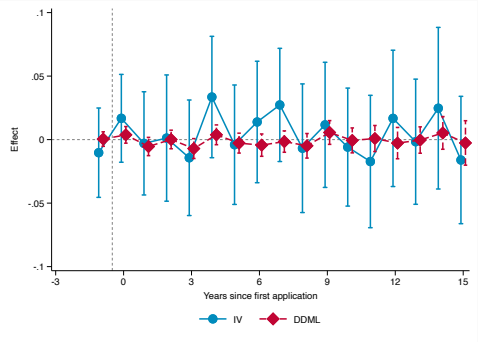
Married



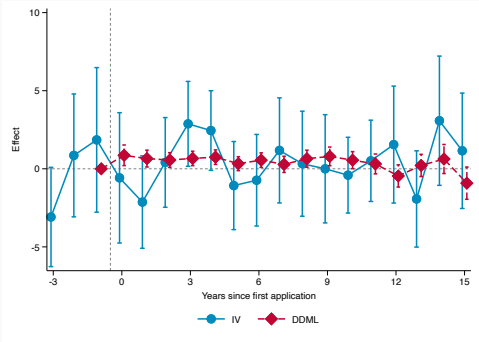
Conclusions

Broader outcomes: crime and hospital nights

Any criminal conviction



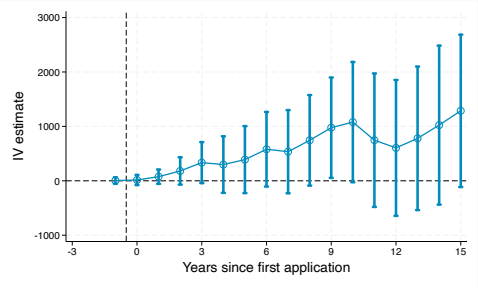
Hospital nights



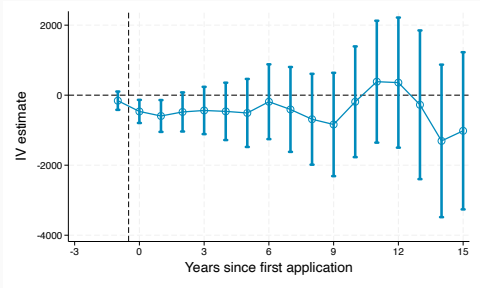
Mechanisms Conclusions

IV: Different welfare benefits

Unemployment Insurance



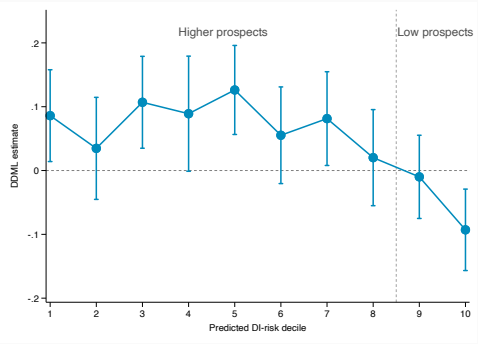
Disability Insurance



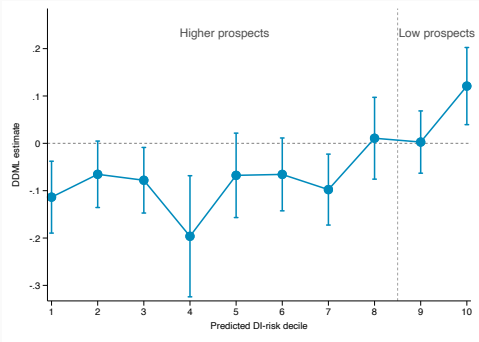
Mechanisms Conclusions

Heterogeneity based on labor market prospects

NEET



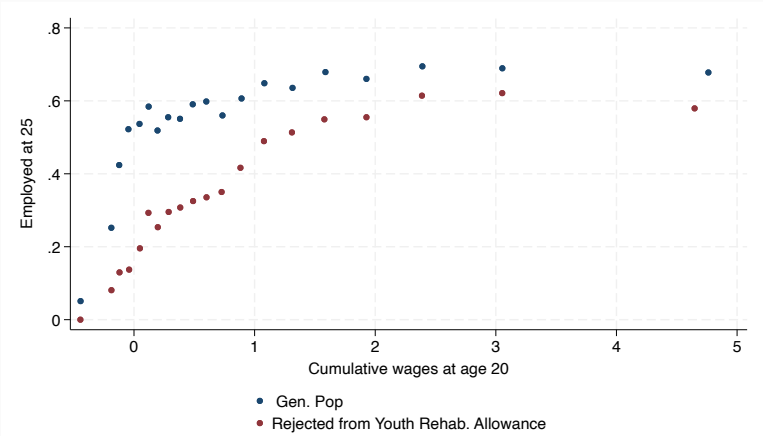
Employed



Conclusions

Work attachment and adult employment

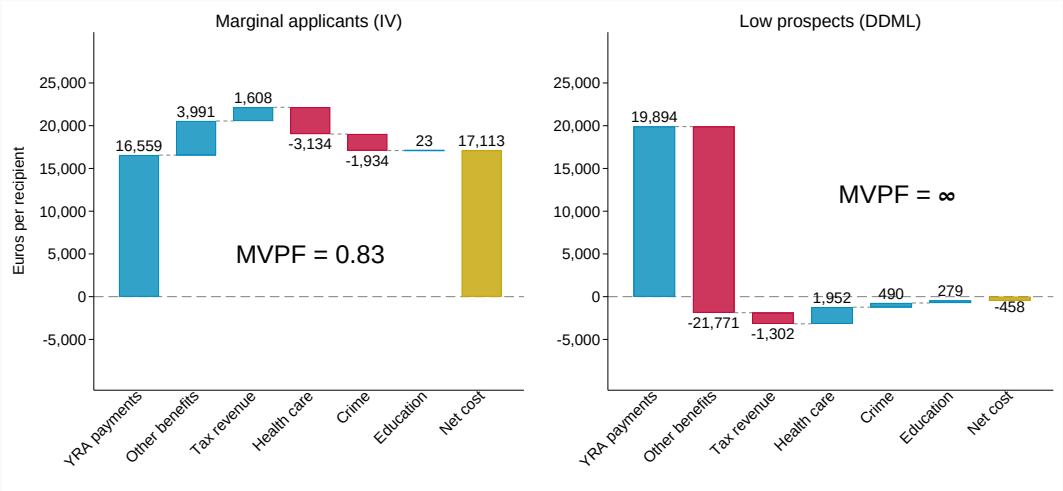
Back



Notes: Wages in €1000. Controls for year, 9th grade GPA, 9th grade GPA missing, and healthcare use at age 16.

The benefit pays for itself only for the low-prospect group

Back



Notes: By age 27. WTP for Marginal applicants 14,281 and for low-prospect 1,613

MVPF

Cost sources

Willingness to pay

The MVPF

Back

$$\text{MVPF} = \frac{\sum_i \text{WTP}_i}{M - FE}, \qquad FE = \underbrace{\Delta\tau}_{\text{taxes}} - \underbrace{\Delta B}_{\text{transfers}} - \underbrace{\Delta H}_{\text{health}} - \underbrace{\Delta CJ}_{\text{crime}}$$

(Hendren and Sprung-Keyser, 2020)

- M: mechanical cost of the transfer at fixed behaviour. FE: what the behavioural responses do to the budget
- The YRA payments enter both sides, so everything I estimate is a deviation from a pure transfer
- Pure transfer: MVPF $= 1$. WTP > 0 and net cost < 0: MVPF $= \infty$

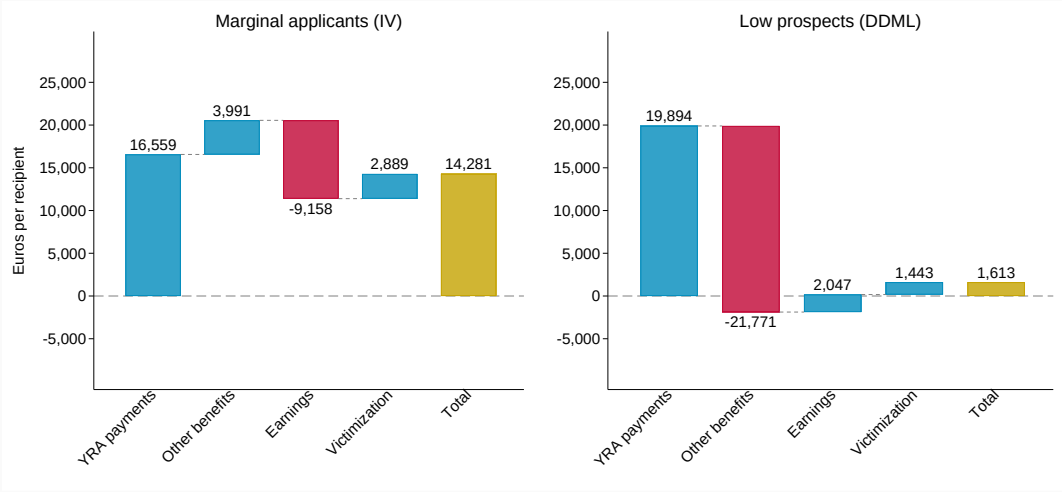
Where the costs come from Back

- Taxes and transfers: FOLK
- Health care: provider costs in the care records, imputed from diagnosis and length of stay where missing. Kela drug costs. No primary care
- Crime: 495 € per investigation, 2 000 € per conviction (Hinkkanen, 2014; Valtioneuvosto, 2026, 2024)
- Victimization: 2 000 € per property crime, 10 000 € per violent or sexual crime, approximated from (Heeks et al., 2018)
- Education: 467 € per student-year of materials and supplies (Opetushallitus, 2025)

Costs winsorized at the 95th percentile, no administrative costs, no effects after age 27

Willingness to pay

Back



Earnings losses during the benefit are an income effect, so only during-benefit losses left out